

STROKE AND TIA REFERRAL FORM

PLEASE INDICATE BELOW WHICH STROKE MEDICINE SERVICE YOU WISH TO ACCESS

	HIGH RISK TIA (Event occurred < 1 week)	LOWER RISK TIA (Event occurred > 1 week ago)
Transient Ischaemic Attack Clinic Referral	☐	☐

Stroke Medicine Clinic Referral	☐
Advice and Guidance from Stroke Medicine Team	☐

To see within 6 weeks

Reply within 48 hours

CONTACT US FIRST FOR TIA REFERRALS

PLEASE CALL THROUGH TO THE **STROKE NURSE PRACTITIONER** ON **07769 932 342 / 07971 717 588**
or the **ON-CALL STROKE CONSULTANT** FOR ALL TIA REFERRALS FIRST SO APPROPRIATE PATHWAY / TRIAGE CAN OCCUR

INCOMPLETE FORMS WILL BE RETURNED

PATIENT DETAILS

Name: Given Name Surname
Address: Home Full Address (single line)
DOB: Date of Birth
NHS Number: NHS Number
Contact Number: Patient Home Telephone Patient Mobile Telephone
Back-up Contact No:

REFERRER DETAILS

Name: Free Text Prompt
Address: Organisation Full Address (single line)
Telephone Number: Organisation Telephone Number
Email address: Organisation E-mail Address

Please supply contact details in case of need for more information to triage or discuss management

ONSET: TIME: DATE:

REFERRAL: TIME: DATE:

PLEASE GIVE A DESCRIPTION OF THE EVENT AND THE QUESTION BEING ASKED

RELEVANT MEDICAL HISTORY AND MEDICATION

See below

STROKE / TIA SYMPTOMS

Unilateral facial weakness	YES ☐	NO ☐
Unilateral arm	YES ☐	NO ☐
Unilateral leg weakness	YES ☐	NO ☐
Unilateral sensory loss	YES ☐	NO ☐
Dysphasia	YES ☐	NO ☐
Visual loss in one eye	YES ☐	NO ☐
Homonymous hemianopia	YES ☐	NO ☐
Diplopia	YES ☐	NO ☐
Ataxia	YES ☐	NO ☐

RISK FACTORS

Atrial Fibrillation	YES ☐	NO ☐
Hypertension	YES ☐	NO ☐
Ischaemic Heart Disease	YES ☐	NO ☐
Smoker	YES ☐	NO ☐
Diabetes Mellitus	YES ☐	NO ☐
Congestive Cardiac Failure	YES ☐	NO ☐
Previous Stroke / TIA	YES ☐	NO ☐
Peripheral Vascular Disease	YES ☐	NO ☐
Hyperlipidaemia	YES ☐	NO ☐
Anticoagulation	YES ☐	NO ☐

NON-STROKE / NON-TIA SYMPTOMS

INVESTIGATIONS DONE

Name: Given Name Surname NHS No: NHS Number

Bilateral central visual loss	YES ☐	NO ☐
Visual aura	YES ☐	NO ☐
Headache	YES ☐	NO ☐
Amnesia	YES ☐	NO ☐
Loss of consciousness	YES ☐	NO ☐
Tingling and numbness	YES ☐	NO ☐
Vertigo	YES ☐	NO ☐

FBC	RESULTS See below
U&Es	
Cholesterol	
Glucose	
ESR	
ECG	

ADVICE FOR REFERRER AND PATIENT

TIA REFERRALS ONLY
- Give 300mg aspirin now and continue daily until seen - Clopidogrel 75mg if aspirin allergic and continue daily - Give atorvastatin 20-80mg if no contraindications - Pulse check to confirm sinus rhythm - Do not drive until seen in TIA clinic - Call 999 if symptoms recur - Ask any witness to accompany patient to clinic

FURTHER INFORMATION
- Ensure you have completed ONSET TIME & DATE - For QEHB send to QEHB.stroke@uhb.nhs.uk - For all HGS sites to HGS.stroke@uhb.nhs.uk - Please expect a follow-up call as more information is often required if a TIA / Stroke mimic is suspected - Expect an appointment within the next seven days or sooner at the LOCAL STROKE CENTRE for TIAs

OTHER REFERRALS	
UNILATERAL VISUAL LOSS:	SIMULTANEOUS REFERRAL TO OPHTHALMOLOGY (BMEC) AND UHB TIA CLINIC IS ADVISABLE
Syncope, Blackouts and/or Falls:	Consider Geriatric Medicine Clinic or Cardiology Clinic Referral
Seizure-Like Episode:	Consider Neurology Clinic / First Fit Referral
Progressive Memory Loss:	Consider Memory Clinic (Via Memory Service At Moseley Hall)

HIGH RISK TIA PATIENTS
The majority of TIA patients are now considered high risk and will be seen soon after referral (often <24 hours) TIA patients that may require admissions include but are not limited to: PERSISTENT NEUROLOGICAL SYMPTOMS FLUCTUATING SYMPTOMS DIFFICULTY SWALLOWING PATIENTS ON FULL ANTICOAGULATION (WARFARIN / DOAC) BLOOD PRESSURE >180/100 CRESCENDO TIAs POSSIBLE DISSECTION CAUSING TIA (FACIAL / NECK PAIN WITH TIA SYMPTOMS)

OTHER INFORMATION
Patients with a suspected acute stroke with ongoing residual symptoms should be conveyed by ambulance to their nearest Hyper-Acute Stroke Unit via 999. This form is for urgent and non-urgent stroke OUT-PATIENTS REQUESTS only within UHB. Please include relevant information from GP records / external hospital records / BMEC records

Problems
Problems

Medication
Medication

Investigations